

Adrabetadex Provider Net Cost Recovery Calculator

Client Concept Review Brief for Beren MLR | Prepared for concept review

Purpose	Explain why Beren needs a provider net cost recovery calculator before launch discussions move into field, ARM, and provider-facing settings.
Core problem	Gross WAC, provider acquisition cost, ASP/payment benchmark, allowed reimbursement, and provider margin are different numbers. Treating them as the same creates reimbursement-readiness risk.
Comparator	Spinraza (nusinersen), used as the closest relevant intrathecal rare-disease analogue for WAC-vs-ASP/payment-limit framing, not clinical equivalence.
MLR posture	Scenario tool for education and planning. Not a guarantee of coverage, payment, acquisition discount, provider margin, or payer behavior.

Bottom line: Beren needs the calculator because Adrabetadex will enter a provider environment where acquisition cost, reimbursement benchmark, site of care, timing, and coding status can each change the provider economic result. A static WAC story does not answer whether the provider is financially whole.

Why This Calculator Is Needed

Client Need	Why It Matters	Calculator Answer
Explain economics beyond WAC	Reviewers and providers may treat list price as the full economic story.	Gap between WAC, net acquisition cost, allowed reimbursement, and provider recovery.
Prepare for provider financial questions	High-cost intrathecal products can create working-capital and payment concerns.	Whether drug cost is recovered after operating cost, denial drag, and payment timing.
Show why 340B status matters	Target COEs may have acquisition economics that differ from WAC.	How the provider result changes when acquisition cost moves below WAC.
Support ARM/account planning	Field teams need a consistent reimbursement discussion without overclaiming.	Which scenarios are workable, thin, or negative under stated assumptions.
Frame the intrathecal comparator	Spinraza shows that WAC alone does not answer provider economics.	How WAC compares with ASP/payment-limit visibility and why assumptions still matter.

What the Calculator Is

Calculator Element	What It Does	Provider-Relevant Output
Acquisition-cost module	Compares WAC acquisition with modeled net acquisition scenarios.	Per-administration acquisition cost and annual exposure.
Reimbursement module	Applies payer/site-specific payment assumptions and WAC-based launch benchmarks where applicable.	Allowed reimbursement and drug spread.
Operating-cost module	Adds labor, pharmacy handling, billing/admin, wastage, denial drag, and financing drag.	Difference between drug spread and true provider recovery.
Scenario module	Compares 340B COE/HOPD, non-340B HOPD, buy-and-bill, white-bagging, and Medicaid pathways.	Scenario-specific surplus, shortfall, or risk flag.
Comparator module	Places Adrabetadex next to Spinraza as an intrathecal analogue.	Separation between WAC, ASP/payment-limit status, net acquisition, and recovery.

Adrabetadex Provider-Economics Framework

Numeric scenarios and intrathecal comparator framing for MLR concept review

Adrabetadex Economics: Why WAC Is Not Enough

Scenario	WAC / Admin.	Modeled Acquisition	Allowed Reimb.	Drug Spread	Operating + Timing	Net Recovery / Admin.	Annualized Net
340B COE / HOPD	\$39,000	\$29,991	\$40,170	\$10,179	\$3,144	\$7,535	\$195,917
Non-340B HOPD / WAC acquisition	\$39,000	\$39,000	\$40,170	\$1,170	\$3,562	(\$1,892)	(\$49,194)
Commercial white-bagging	\$39,000 gross reference	\$0 provider drug acquisition	\$0 provider drug reimbursement	\$0	Administration workflow only	Site-specific	Site-specific

Interpretation: The same Adrabetadex WAC can produce a positive, negative, or site-specific provider result depending on acquisition basis and channel.

Spinraza Comparator: WAC vs ASP / Payment-Limit Visibility

Comparator Layer	Spinraza Public Reference	Implication for Adrabetadex
Relevant analogue	Intrathecal nusinersen, HCPCS J2326, 0.1 mg billing unit	Closest relevant intrathecal rare-disease analogue for provider-economics framing.
Public WAC benchmark	\$125,000 per vial / dose	WAC can be visible and large, but it is not the provider economics result.
Billing-unit structure	J2326 = 0.1 mg; 120 billing units per 12 mg vial	Dose-level price must be translated into billing units before payment logic can be evaluated.
Implied WAC per HCPCS unit	\$1,041.67 per 0.1 mg unit	Claim and payment files operate at HCPCS-unit level.
CMS Q2 2026 ASP/payment-limit status	No J2326 payment limit identified in the April 2026 CMS Medicare Part B Payment Limit File reviewed for this brief	Even an established intrathecal product may not provide a simple public WAC-to-ASP answer.
BuyandBill Q2 2026 ASP display	ASP listed as "Not Available" for Spinraza J2326	Separates WAC, ASP/payment-limit visibility, payer payment, and provider acquisition cost.
First-year gross WAC exposure	\$750,000 using six first-year doses	Exposure depends on both dose price and dosing cadence.
Maintenance-year gross WAC exposure	\$375,000 using three annual maintenance doses	Annual exposure changes over time; a static price point is not enough.

Client-Ready Message Frame

Message	Approved Direction
Why we need it	To show whether providers can recover acquisition cost and operating burden under realistic site, payer, coding, and timing scenarios.
What it is	A provider-economics and reimbursement-readiness tool for scenario planning.
What it is not	Not a clinical value model, coverage guarantee, payment guarantee, pricing recommendation, or claim that a provider receives a specific discount.
Why Spinraza matters	It shows that WAC, ASP/payment-limit visibility, billing units, and provider economics are separate questions.

Source and Assumption Notes

Source / Assumption	Treatment in This Brief
Adrabetadex assumptions	WAC, dosing, annual exposure, 340B proxy value, operating-cost inputs, denial rate, financing rate, and launch timing come from the project workbook and supplied planning assumptions.
Medicare initial sales period benchmark	42 CFR 414.904 allows payment during the initial period, when ASP is unavailable, at an amount not to exceed 103% of WAC.
CMS ASP/payment-limit file	CMS notes that presence or absence of a HCPCS code or payment limit does not indicate Medicare coverage. The April 2026 file reviewed did not identify a J2326 payment limit.
Spinraza comparator	Spinraza PI confirms intrathecal administration and dosing cadence. Public WAC is used only to illustrate WAC-vs-payment visibility and gross exposure.

Sources: 42 CFR 414.904; CMS ASP Pricing Files; BuyandBill Spinraza J2326; CMS Spinraza billing article A58579; Spinraza dosing and prescribing information; BioCentury WAC reporting.